

Institutional Website Redesign Handoff

Replace the retail subscription funnel with a controlled front door for investors, clinicians, healthcare organizations, payers, and security reviewers

Reviewed source	Repository snapshot	Handoff posture
steady-emdr-demo.onrender.com Home, signup, login, crisis, terms, privacy, clinician, audit, and autonomous review views	tprawlins-lang/Emdr Default branch: claude/gifted-keller-501y5d Reviewed commit: dd20ca7	Planning and design only No website or repository changes made

Decision: redesign the public-facing experience now. Keep the calm brand and the strongest product visuals. Remove the consumer purchase path from the active site. Present Steady as a validation-stage behavioral health platform with controlled access to a fabricated demonstration environment.

First release gate: reset and inspect the live demonstration dataset. The current audit console contains personal-looking identifiers and historical login activity that conflict with the site-wide fabricated-data statement. Do not share the environment externally until the audit, member, alert, and event views contain fabricated data only.

Primary audiences: healthcare organizations, clinicians, payers, investors, and security reviewers

Secondary audience: future individual users, with no public enrollment at this stage

Public status: development prototype, not clinical care, not a medical device, not approved for real-person use

1. What changed and why the site must follow

The current website was intentionally written as direct-to-consumer marketing. The source comment in `src/app/page.tsx` describes a retail structure based on outcomes, research, conditions, stories, guarantees, pricing, and a free trial. That matched the first product concept. It no longer matches the next step.

Steady is now being prepared as a broader platform demonstration. The story is no longer only an individual using a self-guided program. It also includes clinical workflow, tenant separation, longitudinal events, governed AI use, payer and organization views, security review, and BLS Part 6 as a separately gated workstream.

The public site must now do five jobs

- Explain the platform in one clear sentence without making clinical or regulatory claims.
- Route each professional audience to the information they need.
- Show enough working product to prove execution without presenting the prototype as approved care.
- Make current, demonstration-only, and planned capabilities visibly different.
- Move every external visitor toward a controlled review request, not a consumer purchase.

The central positioning change

Old site	New site
"Work through trauma, starting today"	"Behavioral health support that continues between visits"
Retail wellness subscription	Validation-stage platform for provider, payer, clinical, and security review
Start free, choose a price, create an account	Explore the platform, choose an audience path, request review access
EMDR method evidence as the lead proof	Product system, human oversight, safety architecture, and evidence status as the lead proof
One member experience	Steady Personal + Steady Clinical + Steady Intelligence
Consumer FAQ	Audience-specific FAQ and Trust Center

2. Current-site audit: what works and what conflicts

Current element	Keep, revise, or remove	Reason
Calm palette, wordmark, rounded cards, generous spacing	Keep	The brand feels steady, human, and distinct from enterprise software.
Persistent demo banner and crisis route	Keep, revise	Keep the boundary. Remove shared credentials and self-service account creation from the banner.
Hero: guided EMDR-based care at home	Replace	It presents a care product for immediate consumer use.
Free week, three pricing tiers, cancel-anytime language	Remove from active site	No retail purchase path should exist during controlled validation.
Composite member testimonials	Remove	They create a consumer-outcome impression before Steady has product-specific clinical evidence.
EMDR trial statistics	Move to Evidence	Method evidence must not look like evidence for Steady itself.
Companion, daily toolkit, safety gate, clinical console visuals	Keep, reframe	These demonstrate product scope when labeled by status.
"Autonomous care loop" and specialist priority claims	Replace	Current governance is shadow/off and real-time clinical coverage is not established.
Signup and subscription routes	Disable or redirect	Use controlled review access instead.
Consumer Terms and Privacy Policy	Replace for T0/T1	They describe subscriptions and real member data, not a fabricated review environment.

Current wording that should not remain on the institutional homepage

- "Starting today" and "begin when you feel ready"
- "A fraction of one traditional session"
- "Everything above is live today"
- "Validated screening" unless the exact instrument and intended use are stated
- "Earlier human review" or "priority specialist review" without an operating team and response standard
- "Safe" as an unqualified product conclusion
- "Care software" where "prototype platform" is more accurate for T0/T1

The issue is not that the current page is poorly designed. It is well designed for the wrong transaction.

3. Immediate sharing gate: clean the demo first

Before the redesigned site points anyone toward the demonstration, run the repository demo reset, recreate the deterministic seed, and inspect every reviewer-facing surface. The banner creates a direct promise. The data must match it.

Check	Pass condition	Evidence
Audit console	No personal email, name, organization, or activity from a real person or prior ad hoc account.	Export or screenshot from the release candidate.
Clinician queue	Every member and clinician carries a visible fictional persona label.	Queue and member-detail inspection.
Alerts and notes	Only scripted synthetic alerts, comments, reasons, and sign-offs remain.	Seed manifest matched to UI.
Companion history	Only seeded synthetic conversations exist.	Database and UI inspection.
Authentication events	Only reserved demonstration identifiers appear.	Audit filter and data scan.
Reset repeatability	Two resets produce the same time-invariant baseline.	Demo baseline and verification command output.
Public access	No route can create uncontrolled accounts or write unscripted personal data.	Route tests and manual review.

Recommended access model

- 1 Public site**
Institutional story, platform status, Trust Center, evidence, and request form. No member enrollment.
- 2 Review gateway**
Invite link or approved access request. Choose investor, clinician, payer, or security review path.
- 3 Fabricated sandbox**
Role-specific personas, guided scenario, reset control, and persistent demonstration labeling.
- 4 Future live application**
Separate host, separate data, separate controls, and separate approval gates. Not part of this release.

Do not expose shared passwords in the global banner. Use expiring links, a review access code, or one-click fabricated persona selection on the review gateway. Keep write access limited by reviewer role.

4. Recommended site and host model

Surface	Purpose	Access posture	Primary action
Public marketing site	Explain the platform and establish trust.	Public, read-only. No account creation.	Request a review or explore an audience page.
Review environment	Show the working synthetic system.	Controlled, fabricated data, named or expiring access.	Start a guided review.
Security room	Share deeper control evidence and known gaps.	Controlled access, read-only package by default.	Request security packet access.
Investor data room	Deck, roadmap, architecture, proof, risks, and use of funds.	Private, tracked access.	Request investor materials.
Future member application	Deliver an approved program to real people.	Not deployed or promoted during T0/T1.	None until T2/T3 gates pass.

Long-term host pattern

Host	Recommended role
www.steady.example	Public institutional website.
demo.steady.example	Fabricated review environment.
trust.steady.example or /trust	Public trust summary with controlled evidence requests.
app.steady.example	Reserved for future approved live use.

Near-term use of the current Render URL

- Treat steady-emdr-demo.onrender.com as the review environment, not the permanent public brand domain.
- Make its root page a review gateway with the demonstration banner, purpose, audience routes, and role-based entry.
- Set robots to noindex and nofollow until the institutional public site is ready and reviewed.
- Remove the retail signup and subscription funnel from all visible navigation and redirects.
- Keep the existing consumer concept in Git history or a named archival tag. Do not leave it on a hidden public route.

5. Public information architecture

Route	Page title	Purpose	Primary CTA
/	Steady Platform	One-page explanation and audience routing.	Request a platform review
/platform	How Steady Works	Personal, Clinical, and Intelligence system story.	View the fabricated demo
/clinical	For Clinical Leaders	Workflow, oversight, safety, summaries, alerts, and pilot questions.	Request clinical review
/organizations	For Healthcare Organizations	Deployment model, care-team workflow, integration, and operations.	Discuss a pilot
/payers	For Payers and Value-Based Care	Engagement, measurement, population signals, and evaluation plan.	Discuss an evaluation
/trust	Trust and Security	Current controls, planned controls, data flows, vendors, and known gaps.	Request security packet
/evidence	Evidence and Validation	Method evidence, product evidence plan, BLS status, and study stages.	Review evidence status
/demo	Review Environment	Persona selection, scenario guides, limitations, and access.	Enter with approved access
/faq	Questions and Answers	Audience-filtered answers with exact current-state wording.	Request a review
/about	Why Steady	Access problem, founder story, product thesis, and team/reviewer needs.	Connect with Steady

Global navigation

Platform | Clinical | Organizations | Payers | Trust & Safety | Evidence | FAQ | Request review | Review sign in

Footer

- Prototype status and no-real-data statement
- Not clinical care, not emergency care, not a medical device, and no public enrollment
- Need immediate help link to the existing crisis route
- Demo Terms, Demo Privacy Notice, Accessibility, Contact, and version/date
- Investor materials in the footer or behind a private request, not as the lead public navigation item

Keep a visible immediate-help link. A professional website about trauma can still receive a visitor in distress. The crisis route remains a safety utility, not a product CTA.

6. Global visual and content system

Element	Direction
Brand	Keep the steady wordmark, olive and sage family, warm ivory background, soft corners, and restrained motion.
Tone	Calm, exact, professional, and human. Avoid clinical coldness and retail urgency.
Hero image	Replace the consumer-at-sunrise illustration with a product-system composition: member experience, clinician queue, and governed data path.
Status labels	Use four visible labels: Working demo , Simulation , In review , and Planned . Never mix them inside one unlabeled feature list.
Product screenshots	Use only fabricated personas. Blur or crop identifiers only as a backup, not as the primary data-control method.
Research	Place method evidence and Steady product evidence in separate sections with separate headings.
Calls to action	Use request review, view platform, discuss pilot, or enter fabricated demo. Remove purchase and free-trial language.
Motion	Quiet transitions only. Honor reduced-motion. Never use urgent or pulsing animation around safety content.
Accessibility	Keyboard navigation, visible focus, semantic headings, contrast, 44px touch targets, text zoom, and screen-reader status labels.

Capability-status component

Label	Meaning	Example
Working demo	Runs in the fabricated environment now.	Clinician queue, audit ledger, deterministic safety simulation.
Simulation	Demonstrates intended behavior without approval for real use.	BLS Part 6 workflow and policy-mode comparisons.
In review	Built or documented, with reviewer decisions still open.	Clinical pilot packet and security findings.
Planned	Roadmap item with no active control or product claim.	Production integration, real payer exchange, live deployment.

Status labels must come from one content object or registry. Do not hand-write status on each page, because drift will create conflicting claims.

7. Homepage handoff

The homepage should answer four questions in the first screen: What is Steady? Who is it for? What can I review today? What is it not yet approved to do?

Recommended hero copy

Eyebrow: Behavioral health platform | Development prototype

Headline: Behavioral health support that continues between visits.

Body: Steady connects structured self-guided experiences, clinician review workflows, and longitudinal behavioral health signals in one reviewable platform. The current environment uses fabricated data and is built for investor, clinical, payer, and security evaluation.

Primary CTA: Request a platform review **Secondary CTA:** See how the platform works

Boundary line: Development prototype. Fabricated data only. Not clinical care, not emergency care, and not approved for real-person use.

Homepage section order

#	Section	Required message
1	Hero and current status	Product definition, audiences, review CTA, and boundary.
2	The gap Steady addresses	Between-visit support, fragmented signals, limited care-team visibility, and access barriers. No outcome promises.
3	Three-part platform	Steady Personal, Steady Clinical, and Steady Intelligence.
4	Working demonstration	Show member, clinician, payer/organization, security, and BLS scenarios with status labels.
5	Human oversight and safety	Deterministic gates, human review actions, alerts, audit, tenant boundaries, and limits.
6	Built for each audience	Four routing cards: Clinical, Organizations, Payers, Security. Investor request in a secondary band.
7	Evidence and validation stage	Separate EMDR method evidence from Steady product validation.
8	Current vs target	Small, honest matrix showing what works, what is simulated, and what still requires review.
9	Final CTA and FAQ preview	Request a review, enter approved demo, or read audience-specific answers.

8. Platform page: the product system

Product layer	Public description	What to show	Status wording
Steady Personal	Structured between-visit experiences, check-ins, grounding, education, companion support, and progress signals.	Member dashboard, daily plan, grounding, check-in, companion, consent and memory controls.	Working demo; no public enrollment.
Steady Clinical	A clinician-facing workflow for caseload, timelines, alerts, cited summaries, review, correction, override, and re-entry.	Clinical queue, synthetic member detail, alert ownership, audit, policy comparison.	Working demo plus in-review workflows.
Steady Intelligence	The event, policy, audit, tenancy, and analysis layer that connects member and clinical activity over time.	Event spine, projection replay, tenant controls, audit console, capability status.	Working foundation; some controls dormant or planned.

Platform story sequence

- **Observe:** a member creates structured signals through check-ins, exercises, conversations, and sessions.
- **Protect:** deterministic rules evaluate fit, readiness, safety, modality, and current state before model use or session access.
- **Record:** versioned events and audit entries preserve who did what, under which policy and code version.
- **Review:** clinicians see a synthetic caseload, cited summaries, alerts, and separate approve, correct, and override actions.
- **Learn:** authorized organization and payer views use fabricated aggregate signals to demonstrate evaluation possibilities.
- **Govern:** policy modes, tenant boundaries, security evidence, and current-vs-target labels stay visible.

What not to show as a finished platform capability

- Real-time clinician monitoring
- Approved AI clinical summaries
- Production payer exchange or reimbursement
- Production HIPAA compliance
- Validated clinical outcomes from Steady
- Autonomous real-person BLS or trauma processing

9. Clinical page handoff

Page goal: let a clinical leader understand the intended workflow, see exactly where people remain accountable, and identify the questions that need clinical review.

Section	Content
Hero	"A clinician review layer for between-visit behavioral health support." State that this is a fabricated prototype.
Caseload	Clinical-need ordering, primary owner plus coverage pool, alert deadlines, no immediate-band auto-resolution.
Member timeline	Source events, reconstructed history labels, policy version, and model/human separation.
AI summaries	Citations, uncitable-claim suppression, uncertainty, and separate approve, correct, and override actions.
Safety and coverage	Deterministic pause, crisis resources, grounding, coverage-hours language, and no promise of 24/7 monitoring.
Configurable policies	Companion content access, caseload ownership, coverage, alert consequence, re-entry, and autonomous mode.
BLS Part 6	Active validation workstream, shown only as a fabricated simulation; real use remains separately gated.
Review request	Invite clinical reviewers to assess workflow, copy, boundaries, alert duties, and pilot prerequisites.

Clinical CTA language

Primary: Request a clinical workflow review

Secondary: View the fabricated clinician scenario

Do not use: Start treating, enroll patients, monitor your patients, or deploy now

Preserve the prior consumer review record. State that beta-clinrev-2026-07 did not cover the clinician surface, multi-tenancy, clinical-record summaries, or the new operating model. Point professional reviewers to clinical-pilot-2026-09 as the proposed review packet.

10. Healthcare organization and payer pages

Page	Lead message	Sections	Claims boundary
Organizations	A reviewable platform for structured support between visits and clearer care-team signals.	Operating model; clinical workflow; member engagement; integration path; deployment stages; governance; pilot discussion.	Do not claim reduced workload, improved outcomes, deployment readiness, or existing integrations without evidence.
Payers	A measurement and engagement layer designed for value-based behavioral health evaluation.	Population questions; engagement measures; longitudinal signals; evaluation design; organization workflow; data exchange roadmap.	Do not claim cost savings, reimbursement, coverage, ROI, utilization reduction, or validated risk prediction.

Healthcare organization page detail

- Show how a synthetic organization is created and separated from another tenant.
- Explain the primary-owner and coverage-pool model as a configurable demonstration assumption.
- Show the operating roles required for a pilot: clinical owner, security owner, privacy/legal review, support, and incident response.
- Describe EHR/FHIR, identity, and data-exchange work as target architecture unless it exists on the reviewed commit.
- Offer a pilot-design conversation, not deployment.

Payer page detail

- Use fabricated population summaries and clearly separate aggregate views from individual clinical records.
- Frame measures as candidates for evaluation, not proven leading indicators.
- Describe potential payment lanes broadly: partner-sponsored, payer-supported, value-based, or self-pay. Do not name a billing code as settled.
- Show the validation questions: adoption, engagement, safety events, clinical usefulness, retention, utilization, and cost measurement.
- Offer an evaluation-design discussion with agreed measures and data boundaries.

11. Trust Center and Evidence pages

These pages should earn confidence through precision. They should not read like security or clinical certification pages.

Trust Center section	Required content
Current posture	Fabricated data only; no real-person use; no production healthcare claim.
System and data flow	Six governance zones, trust boundaries, stores, vendors, model egress, and administrative paths.
Control status	Current, dormant, and planned controls separated. Include tenant repository adoption and RLS activation status.
AI and vendors	Commercial model provider, exact content sent, retention/contract/BAA status, and controls before model calls.
Identity and tenancy	Roles, tenant context, Postgres RLS status, attack tests, and remaining request-path work.
Audit and monitoring	Event families, tamper evidence, alert ownership, logging limits, and reviewer export.
Known gaps	Named findings, owner, target tier, mitigation, and acceptance test.
Request packet	Controlled access to threat model, risk register, vendor/BAA register, incident plan, and test evidence.

Evidence page structure

Evidence class	How to present it
Evidence for the EMDR method	Published research and guidelines, clearly stated as evidence about clinician-delivered EMDR, not proof of Steady.
Evidence for Steady software behavior	Tests, deterministic replay, safety scenarios, tenant attacks, accessibility checks, build and scan results.
Evidence still needed	Human-factors work, clinician workflow review, security audit, pilot protocol, product outcomes, and payer evaluation.
BLS Part 6	Protocol and validation status, simulated behavior, open review needs, and real-use exclusion.

Never use "HIPAA compliant," "clinically validated," "secure," or "approved" as a general label. Use dated evidence and exact scope instead.

12. Review gateway, login, and access

The current signup and login pages still present consumer prices, a free trial, and shared credentials. Replace them with a review gateway.

Current route	New behavior
/signup	Redirect to /demo or /request-review. No public account creation.
/subscribe	Disable in T0/T1. Return a clear prototype-stage page or redirect to review request.
/login	Rename visually to "Review sign in." Remove consumer language and shared passwords.
/demo	New role and scenario gateway with access rules, dataset version, limitations, and reset status.
/crisis	Keep public and available without authentication.
/dashboard and /clinician	Require approved review access. Preserve the persistent fabricated-persona banner.

Review gateway sequence

1

Choose review purpose

Investor overview, clinical workflow, organization/payer view, or security audit.

2

Read the boundary

Fabricated data, no clinical care, no real-time monitoring, and no public enrollment.

3

Select a persona and scenario

Member, clinician, organization analyst, payer reviewer, or security reviewer. Scope permissions to the role.

4

Enter the guided sandbox

Show scenario steps, expected states, status labels, reset version, and a feedback path.

Access controls

- Use expiring access or approved invitations for write-capable roles.
- Default investor and payer access to read-only guided views.
- Do not expose raw audit identifiers to general demo viewers.
- Separate security evidence access from ordinary product-demo access.
- Log review access using reserved fabricated identifiers only.

13. FAQ redesign: structure

Replace the six consumer questions with grouped questions that answer the concerns of professional reviewers and prevent overstatement.

FAQ group	Questions to include
Platform	What is Steady? What parts are working today? What is simulated? Who is the platform designed for?
Clinical	Is Steady therapy? Are clinicians monitoring activity? Does AI make care decisions? How are alerts handled? What is BLS Part 6?
Organizations	How would a pilot work? What roles are required? Does Steady integrate with an EHR? Can organizations configure policies?
Payers	What can Steady measure? Is reimbursement established? Are savings proven? How would an evaluation be designed?
Security and privacy	Does the demo contain PHI? Is Steady HIPAA compliant? What data reaches the model provider? Are BAAs in place? How is tenant isolation tested?
Access and stage	Can individuals sign up? Can we use real patient data? How do we review the demo? When will production use begin?
Evidence	Is Steady clinically validated? What evidence supports EMDR? What product evidence exists? What review is still needed?

FAQ behavior

- Use a dedicated /faq page with audience filters. Show six to eight top questions on the homepage.
- Lead every answer with the direct answer: Yes, No, Not yet, or In the fabricated demo only.
- Add last-reviewed date and owner to clinical, security, privacy, and legal answers.
- Link answers to the matching Trust Center, Evidence, or workflow section.
- Do not use FAQ copy to make claims that the main page avoids.
- Keep a visible immediate-help answer and link to /crisis.

The FAQ is part of the control system. It should use the same status registry and claims language as every other public page.

14. FAQ redesign: approved draft answers

Question	Draft answer
What is Steady?	Steady is a development-stage behavioral health platform that combines structured between-visit experiences, clinician review workflows, and longitudinal signals. The current environment is a fabricated demonstration, not clinical care.
Can individuals sign up?	Not at this stage. Public enrollment and subscription billing are closed while Steady is prepared for clinical, security, privacy, and partner review.
Is Steady therapy or medical care?	No. The current environment does not provide therapy, medical care, diagnosis, treatment, or emergency services.
Are clinicians monitoring the demo?	No. The demonstration shows a proposed clinician workflow, but no one monitors activity in real time and no care team is assigned.
Does AI make clinical decisions?	No in the current demonstration posture. Deterministic rules control safety and access behavior. AI-generated summaries or suggestions must be labeled and reviewed by a person. The newer autonomous engine remains off or in shadow mode.
Does the demo contain PHI?	It must not. The review environment is restricted to fabricated personas and scripted data. Any real-person information is a stop condition.
Is Steady HIPAA compliant?	Steady does not make that claim today. The team is building and testing technical controls, while contracts, BAAs, operating procedures, independent review, and production deployment controls remain required before real healthcare use.
What data reaches the AI provider?	Companion messages, recent conversation history, and relevant program context may be sent to the commercial model provider to generate a response. The Trust Center should list the exact fields, contract state, and controls.
What is BLS Part 6?	It is an active validation workstream. The fabricated demo may simulate the workflow, including stop and escalation behavior. It is not approved for real-person use.
Is Steady clinically validated?	Not yet. Published evidence about clinician-delivered EMDR supports the method, not Steady itself. Steady still needs human-factors work, clinical review, security review, and staged product evaluation.
Is reimbursement available?	No reimbursement path is presented as established. Steady is exploring partner-sponsored, payer-supported, value-based, and self-pay models with reviewers.
How can my organization review Steady?	Request a review and choose the clinical, organization, payer, investor, or security path. Approved reviewers receive a guided fabricated scenario and the matching evidence packet.

15. Claims and copy policy

May say	Must qualify	Do not say
Development prototype	Built for review	Ready to deploy
Fabricated demonstration	Architected for healthcare use	HIPAA compliant
Deterministic safety rules are implemented and tested	Tenant and RLS controls are built; state whether they are active on the request path	Tenant isolation is fully enforced
AI-assisted summaries in the demo	Human review is intended or required in the workflow	AI clinical decision-making
EMDR method evidence	Studies describe trained-clinician delivery	Steady outcomes are proven
BLS simulation	Separate real-use gates remain	Autonomous EMDR is approved
Proposed clinician workflow	Coverage and accountability remain under review	A clinician is always watching
Evaluation pathway	Potential payer and partner models	Covered, reimbursed, or proven to save money

Automated copy guard

Add a public-copy test that scans active public pages for retired consumer CTAs and restricted phrases. The test should allow approved qualifications in controlled contexts, not rely only on a raw banned-word list.

- Fail if active public pages link to /signup or /subscribe.
- Fail if pricing, free trial, consumer enrollment, or purchase copy appears in T0/T1 public routes.
- Fail if "HIPAA compliant," "clinically validated," "approved care," or "24/7 monitoring" appears without a test-approved exception.
- Fail if a screenshot fixture or seed contains an identifier outside the reserved demo patterns.
- Fail if any capability card lacks a current status label.
- Fail if the demo boundary is absent from public, login, clinician, payer, and security review layouts.

Keep claims in one versioned content registry with owner, status, evidence link, last-reviewed date, and permitted audience. Pages should read from that registry.

16. Terms, privacy, and trust copy

The existing Terms of Service and Privacy Policy describe consumer subscriptions, real wellness data, deletion, refunds, and member use. They should not remain the governing documents for a fabricated institutional review environment.

Document	T0/T1 replacement
Demo Terms of Use	Evaluation-only license, no clinical reliance, no real-person data, no credential sharing, no production use, feedback handling, intellectual property, access termination, and liability language reviewed by counsel.
Demo Privacy Notice	Data collected from reviewers, access logs, feedback, cookies, hosting, retention, security contact, and a clear ban on submitting personal health information.
Public Privacy Notice	Public website telemetry, forms, communication, retention, and no advertising-tracker statement only if technically verified.
Trust Center statement	Dated current controls, dormant controls, planned controls, vendors, model egress, BAA status, known gaps, and security contact.
Accessibility statement	Current accessibility testing, supported browsers, contact route, and known issues.

Required copy distinction

Public site	Fabricated demo	Future real-person service
Explains Steady and collects review requests.	Collects only reviewer access and scripted synthetic activity.	Requires separate privacy, consent, clinical, security, contracting, and operating review.
No health-data intake.	No real health-data intake.	Not authorized by this handoff.

Counsel should review the new terms and privacy notices before publication. Engineering must still confirm that every factual privacy and security sentence matches the release candidate.

17. Repository implementation map

The coding agent must begin by confirming the live branch, exact HEAD, open work, current routes, deployment commit, and demo reset state. The paths below are based on commit dd20ca7 and may move.

Path	Action
src/app/page.tsx	Replace the direct-to-consumer page with the institutional homepage.
src/app/layout.tsx	Update title, description, robots posture, global banner, footer, and institutional metadata.
src/app/signup/page.tsx	Remove public enrollment behavior; redirect to /demo or /request-review.
src/app/subscribe/	Disable retail subscription access during T0/T1.
src/app/login/page.tsx	Convert to Review Sign In. Remove shared credentials and consumer copy.
src/app/terms/ and src/app/privacy/	Replace with demo and public-site documents after counsel review.
src/app/crisis/	Retain public access and current emergency boundaries; verify country handling.
src/app/clinician/	Keep behind review access; add audience guide, role permissions, and fabricated-data verification.
New public routes	Add platform, clinical, organizations, payers, trust, evidence, demo, FAQ, about, and request-review pages.
src/components/Brand.tsx	Reuse the wordmark and mark.
src/components/Illustrations.tsx	Reuse selectively; add product-system and audience illustrations.
New shared components	PublicHeader, PublicFooter, StatusBadge, AudienceCard, ProductLayer, TrustControl, EvidenceClass, ReviewCTA, and FAQGroup.
New content registry	Versioned public claims, capability status, evidence links, owners, and review dates.
Tests	Add public posture, route, claim, access, fabricated-data, accessibility, mobile, and navigation coverage.

Do not rebuild working platform capabilities

The redesign should read existing status, safety, policy, and demo fixtures where possible. It should not duplicate event, tenancy, clinical policy, evidence, or audit logic inside marketing components.

18. Build sequence and release acceptance

Phase	Work	Exit condition
0. Confirm and clean	Repository preflight, deployment SHA, dataset reset, identifier scan, route inventory.	Fabricated-only demo verified.
1. Change posture	Metadata, robots, banner, navigation, retail route removal, review sign-in.	No public purchase or enrollment path.
2. Build core pages	Home, Platform, Clinical, Organizations, Payers.	Audience story and status labels approved internally.
3. Build trust pages	Trust Center, Evidence, FAQ, About, Demo gateway.	Claims match code and packets.
4. Add access flow	Review request, role selection, guided scenarios, permissions.	Each audience reaches the right read-only or review surface.
5. Rewrite legal copy	Demo Terms, Demo Privacy, Public Privacy, accessibility statement.	Counsel and engineering factual review complete.
6. Test and release	Desktop/mobile, keyboard, screen reader, routes, data scan, claims scan, reset, links.	Release checklist passes on exact deployment commit.

Release acceptance checklist

- No real-person identifier, health information, or unscripted historical activity appears anywhere in the review environment.
- No public route offers a free trial, retail price, subscription, or uncontrolled account creation.
- The homepage identifies Steady as a development prototype within the first screen.
- Every capability shown to reviewers carries Working demo, Simulation, In review, or Planned status.
- Clinical, payer, security, and product claims match the exact deployment commit and evidence index.
- Public and demo terms describe the actual T0/T1 data flows and access model.
- The crisis route stays public and works without login.
- All audience paths work on mobile and desktop with keyboard and screen-reader navigation.
- The review gateway uses role-limited access and no shared credentials in the global banner.
- The release record includes deployment SHA, dataset version, reset proof, tests, reviewer, and date.

Final direction: keep the warmth, lose the retail urgency. The site should make Steady look larger because the platform is larger, but it should also make the stage clearer. A visitor should leave knowing what works, what is simulated, what still needs review, and how to inspect the system without mistaking a strong demonstration for approved clinical use.